

CORTISONE CREAM

VICOR01-PC2 (MY)

DESCRIPTION

White, opaque cream.

COMPOSITION

Hydrocortisone 1% w/w.

PHARMACODYNAMICS

Hydrocortisone diffuse across cell membranes and complex with specific cytoplasmic receptors. These complexes then enter the cell nucleus, bind to DNA (chromatin), and stimulate transcription of messenger RNA (mRNA) and subsequent protein synthesis of various enzymes thought to be ultimately responsible for antiinflammatory effects of topical application of hydrocortisone.

PHARMACOKINETICS

Hydrocortisone is well absorbed through the skin after topical application. After an intravenous dose of 100mg as a soluble compound, plasma concentrations of up to 1ug/ml are attained in 30 minutes. Plasma half life is 80 to 120 minutes. This may be increased in patients with liver disease, hypothyroidism, or in patients undergoing oestrogen therapy.

Hydrocortisone is widely distributed throughout the body. About 90% of hydrocortisone is bound to plasma proteins, mainly to corticosteroid binding globulin and also to albumin. Hydrocortisone is metabolised in the liver and other tissues by ring-A reduction, 20-ketone reduction, side-chain cleavage, ring-A hydroxylation, and conjugation with glucuronic acid or sulphate.

About 90% of a dose is excreted in the urine in 24 hours. Less than 1% is excreted unchanged, 40% is the 5 α - and 5 β -forms of tetrahydrocortisol and the 5 β -form of tetrahydrocortisone. 17% is a mixture of the 5 α - and 5 β - and 20 α - and 20 β -cortols and cortolones, 6% is 11-hydroxy and 11-keto-etiocholanoline. Most of the metabolites are excreted as conjugates; at plasma concentrations of greater than 0.2ug/ml, the amount of unchanged drug excreted in the urine is increased.

INDICATIONS

For relief or treatment of:

- Minor skin irritation, itching, and rashes due to eczema, insect bites, poison ivy, oak, or sumac dermatitis, detergents, cosmetics and jewellery.
- Anogenital pruritus.

CONTRAINDICATIONS

- Excessive or prolonged usage in pregnancy is not recommended.

- Avoid contact with eyes.
- It should not be applied to ulcers of the leg.

PRECAUTIONS

- Caution in patients with infection at treatment site, and tuberculosis of the skin.
- Paediatric therapy continuing for longer than 2 weeks should be evaluated carefully by the physician.
- Do not bandage or otherwise wrap the treated skin area unless directed to do so by physician.
- Safety for use in lactation has not been established.

MAIN SIDE/ADVERSE EFFECTS

Increase liability to infection; prolonged use include moon face, glaucoma, thinning of skin with easy bruising, increase in hair growth, loss of hair, reddish purple lines on arms, legs, trunk, or groin.

Note: It has fewer side-effects on the skin than the more potent topical corticosteroids.

OVERDOSAGE

Clinical features:

Atrophic changes in the skin, secondary infections, burning, itching, irritation, dryness, folliculitis, hypertrichosis, acne, and pigmentation.

Treat overdosage by symptomatic measures, where possible the dosage being reduced or the drug slowly withdrawn.

DOSAGE AND ADMINISTRATION

Apply sparingly to the affected area.

Adults : Three or four times a day.

Children : One or two times a day.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage: Store below 30°C. Protect from freezing.

Presentation/Packing: Cream 1% x 5 g and 15 g.

Product Registration Holder / Manufactured by: HOVID Bhd.
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Revision date: August 2017

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